INFORMATION TO KEEP WITH YOU

# My complete treatment handbook

## Panitumumab

Understand your treatment. Know what to expect. Keep your plan to hand.

**Displayed calendar**
**14 days**
See the active treatment phase

**Treatment stages**
**1**
Explained in the treatment reference

**Regimen source**
**NCCP 00225**
Version 6

> Panitumumab on Day 1 every 14 days while beneficial and tolerable. There is no home chemotherapy pump.

## Treatment stages

01 **A repeating 14-day cycle**
Infusion → home → review

Your team confirms the active stage and dates. The full treatment guide explains each step.

## Inside your handbook


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**My name** _____

**Hospital / oncology team** _____

> Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

# Understanding your treatment

Panitumumab · National Cancer Control Programme (NCCP) 00225

## When to get help

**Emergency: call 112 / 999**

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

**Contact your oncology service immediately**

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report new breathing difficulty, eye pain or blurred vision, blistering or infected skin, severe diarrhoea or dehydration promptly.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

## How your treatment works

The aim is to control advanced bowel cancer and symptoms. The team reviews how well treatment is working and whether side effects remain acceptable; there is no single fixed number of cycles for everyone.

EGFR-TARGETED ANTIBODY

### Panitumumab

Vectibix



**Blocks growth messages at the cell surface**

Panitumumab attaches to EGFR, a receptor on the outside of cells. This can interrupt signals that help some bowel cancers grow. A tumour RAS test helps identify whether this approach is suitable. EGFR also supports healthy skin, which explains why skin and nail changes need attention.

EGFR receives a growth signal → Panitumumab blocks the receptor → Some growth signals are reduced

**What to watch for**

Report rash, painful cracks or nail changes early. Eye pain or blurred vision, new breathing symptoms, severe diarrhoea, or blistering skin need prompt assessment. Blood tests check minerals such as magnesium.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

## Your treatment aim and review plan

Understand why this option fits your cancer.

### The setting matters

Panitumumab is used for bowel cancer that has spread and has a confirmed wild-type RAS result. It is given after earlier fluoropyrimidine, oxaliplatin and irinotecan treatment has failed. Your team explains the aim, alternatives and how benefit will be reviewed.

### What the tumour tests mean

RAS wild type is required for panitumumab. Your team also considers the cancer location, other molecular results, previous treatment and whether surgery or other local treatment could be useful. A positive eligibility result is not a promise of benefit.

## 20100007: panitumumab monotherapy

Studies can help explain why a treatment is offered. The schedule, previous medicines and tumour tests affect how well the evidence fits you.

Panitumumab improved survival in a trial group selected using tumour testing after other bowel-cancer medicines had stopped working. The likely benefit for you depends on your cancer, previous treatment and general health; it cannot be predicted from the trial alone.

SACTCHECK · CHAPTER 02

# Your schedule, tests and appointments

Panitumumab · NCCP 00225

## Treatment, review and the next cycle

**Panitumumab**

14-day view




This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

INFUSION → HOME → REVIEW

### A repeating 14-day cycle

D1 D14

Day 1: panitumumab infusion. There is no home chemotherapy pump in this regimen. A cycle lasts 14 days. Treatment continues while it is helping and side effects remain acceptable.

A repeating reference cycle sits within a longer treatment plan. The oncology team chooses the number of cycles and confirms changes; no patient dates are required here.

BASELINE → TREATMENT → FOLLOW-UP

## When checks happen

The columns show order, not equal periods. There is no fixed number of cycles for these plans. Agree who books each check, when results are reviewed and whom to contact if a check is missed.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00225 v6 · official regimen · Panitumumab product information

## Tests in your plan

### Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

**For a colorectal cancer plan, if your team uses CEA; your team sets the interval.**

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

# Panitumumab: treatment and checks

## Advanced / metastatic colorectal cancer

Panitumumab on Day 1 every 14 days while beneficial and tolerable. There is no home chemotherapy pump.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | Early treatment | Repeated treatment | Longer-term review | Pause / completion |
|---|---|---|---|---|---|
| **Blood counts, kidney and liver** NCCP 00225 | Baseline blood count, kidney and liver profile | Review results before treatment | Blood count, kidney and liver tests before each treatment | Repeat sooner if symptoms or results need it | Follow any unresolved abnormal result |
| **Magnesium, calcium and potassium** NCCP 00225 | Check mineral levels before starting | Repeat periodically during treatment | Keep blood-test and replacement appointments | More checks may be needed if levels fall or diarrhoea develops | Monthly checks for two months after the last antibody dose |
| **Skin, nails, eyes and breathing** NCCP 00225 | Discuss existing skin, eye or lung problems | Start the prescribed skin-care plan; report reactions early | Review skin, nails, diarrhoea and new symptoms | Eye pain, worsening breathing or severe rash needs prompt assessment | Continue care until symptoms and mineral levels settle |
| **Cancer response and treatment choices** NCCP 00225 | Record baseline disease and agree review plan | Bring forward review if symptoms worsen | Scans and other tests follow your consultant's treatment plan | Discuss benefit, side effects and whether the goal has changed | Agree continuation, a break, another treatment or supportive care |

## What each check is for

### 1 A repeating 14-day cycle

Infusion → home → review

The aim is to control advanced bowel cancer and symptoms. The team reviews how well treatment is working and whether side effects remain acceptable; there is no single fixed number of cycles for everyone.

**Ask / act:** Confirm the current phase, prescription, tests and support medicines.

**At the next review, ask**

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

## Visits, access and care between treatments

STEP 1

### Before the first visit

Confirm the exact regimen, tests, medicines, emergency number and treatment goal. Ask whether a peripheral cannula or an existing line is appropriate; a port is not automatically required.

STEP 2

### At the treatment unit

The first panitumumab infusion usually takes about one hour; higher doses or reactions may need longer. Later tolerated infusions may be shorter. Blood review, preparation, premedicines and observation add time; infusion time is not the total appointment length.



STEP 3

### After the infusion

There is no fluorouracil home pump and no disconnection visit in this regimen.

STEP 4

### Between doses

Use the written symptom and support plan; do not wait for the next appointment if urgent symptoms appear.

STEP 5

### The next decision

Confirm the next visit after any delay or dose change. Treatment continues only while the oncology team judges benefit and tolerability acceptable. Follow-up mineral checks continue after the final EGFR-antibody dose where applicable.

#### Transport and daily life

Plan transport if antihistamines, symptoms or other medicines affect driving. Ask for practical, work, nutrition or emotional support.

### If treatment changes

Ask which component has been held and which visits remain. A treatment delay does not automatically cancel blood tests, line care or symptom follow-up.

## Before you start and before each cycle

The team checks that the plan remains suitable.

### Checks and medicines

Review blood counts, kidney/liver function and minerals, current symptoms, allergies and all medicines. dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) testing is not a requirement for panitumumab alone.

### Skin, eyes and breathing

Tell the team about an existing rash, skin infection, dry or painful eyes, contact lenses, or previous lung inflammation. Report new eye pain, light sensitivity or visual change promptly.

## When treatment changes or finishes

Follow-up continues after the last infusion.

### Monitoring and recovery

Continue your skin plan and planned magnesium, calcium and potassium checks for about eight weeks after panitumumab. New symptoms still need review. Ask when to stop prescribed supportive medicines.

### Treatment choices

Your team may pause, reduce, change or stop one or more medicines. Ask which medicine changes, why, and what the next review will assess. Supportive and palliative care can help alongside active treatment.

SACTCHECK · CHAPTER 03

# Symptoms and supportive care

Panitumumab · NCCP 00225

## Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

| Treatment or check | Timing in this view |
|---|---|
| **Panitumumab infusion** |  **Day 1** intravenous (IV) |
| **Skin / nails** |  **Days 1–14** May develop or accumulate over cycles |
| **Bowel / fatigue** |  **Days 1–14** Timing varies |
| **Mineral changes** |  **Days 1–14** Blood tests during and after treatment |

**The planned treatment:** Day 1: panitumumab infusion. There is no home chemotherapy pump in this regimen. A cycle lasts 14 days. Treatment continues while it is helping and side effects remain acceptable.

**Blood tests after the final dose:** Mineral checks continue for about eight weeks after panitumumab. There is no guaranteed safe day for symptoms.

**Skin care continues between visits:** Rash or nail changes may build over several cycles. Report symptoms early and follow your written plan.

**Why these bands stay broad:** Bars show planned treatment and possible symptom patterns, not measured risk. Side effects can overlap and persist. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00225 v6 · official regimen ↗ · Panitumumab product information ↗

**There are no guaranteed "safe days".** Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

**Between-treatment notes**

### Day 1: Review and infusion

The team checks symptoms, blood tests and treatment suitability, then gives panitumumab through a vein. Tell the nurse immediately about infusion symptoms. Confirm the skin-care plan, contact number and next visit.

### Day 2–7: At home

There is no chemotherapy pump to disconnect. Watch skin, bowel and eye symptoms and use the prescribed care plan. Report changes early; serious symptoms can happen on any day.

### Day 8–14: Continue care and prepare for review

Rash, nail changes and mineral problems may build over several cycles. Feeling well does not replace blood tests. Bring your symptom record. Do not wait for the next infusion if symptoms are concerning.

## Support around the medicines

**1** **Start skin care early**

Ask for your skin-care plan before the first infusion. Use the recommended moisturiser and sun protection; your team may prescribe a cream or an antibiotic. Do not use ordinary acne treatments without advice.

**2**  **Minerals and blood tests**

Panitumumab can lower magnesium and other minerals even when you feel well. Blood tests continue during treatment and for about eight weeks after it finishes. Take replacement treatment only as prescribed.

**3** **Feeling sick or loose stools**

Panitumumab has low nausea risk, but symptoms can still occur. Follow the rescue medicine and hydration plan from your team. Severe or persistent diarrhoea, reduced urine or inability to drink needs urgent advice.

**4** **Travel and daily life**

Allow time for blood tests, review, preparation, infusion and observation. Ask about transport if medicines or symptoms affect driving. Pace activity and seek help with food, work or caring responsibilities.

**5** **The whole treatment plan**

Ask how and when the team will assess benefit, which blood tests continue during a pause, and what happens after the final dose. Mineral checks continue monthly for two months after the last antibody treatment.

## Living with treatment

Build the plan around what matters to you.

### Skin and nails

Ask for your skin-care plan before the first infusion. Use the recommended moisturiser and sun protection; your team may prescribe a cream or an antibiotic. Do not use ordinary acne treatments without advice. Avoid friction on sore skin and tell the team about painful nail folds.

### Food, activity and support

Keep fluids and nourishing food within reach. Pace activity around energy levels and ask for dietetic, emotional or practical support. If symptoms prevent eating, drinking or usual tasks, contact the team.

### Pregnancy and fertility

Discuss pregnancy plans and fertility before treatment. Panitumumab requires contraception during treatment and for two months afterwards, and breastfeeding should be avoided for that period. When chemotherapy is also included, its precautions may last longer; ask for the instructions for the whole regimen.

## Recognise changes early

You do not need to identify the responsible medicine before asking for help.

### Track impact, not just a score

Describe when symptoms began, whether they are worsening, and whether they affect food, fluids, sleep, walking or daily tasks. Your team assigns the clinical grade. Use the passport if it helps; it is not required to access this information.

## Visits, access and coordination

Know what happens in the clinic and what happens at home.

### The delivery plan

Day 1: panitumumab infusion. There is no home chemotherapy pump in this regimen. A cycle lasts 14 days. Treatment continues while it is helping and side effects remain acceptable.

### Radiotherapy or other treatment

This regimen has no routine radiotherapy component. If another treatment is proposed, ask whether it happens before, during or after this course and who will coordinate the appointments.

## Questions worth asking

### Why do I need a RAS test?

RAS is part of the growth-signalling pathway inside cancer cells. If it is changed by a mutation, blocking EGFR is unlikely to help. Your team needs a validated wild-type RAS result before panitumumab. Other tumour tests and the treatment aim also affect the choice.

### Is this the same as checkpoint immunotherapy?

Panitumumab is a targeted antibody against EGFR. It works differently from checkpoint treatments used in some other cancer regimens. Ask your team which treatment class and side effects apply to you.

### Does a rash mean the treatment is working?

A rash is a recognised drug effect, but it cannot tell you whether your cancer is responding. Report it early so it can be treated. Scans, symptoms and clinical review guide the response assessment.

### Will I need radiotherapy as part of this cycle?

These panitumumab regimens do not contain a scheduled radiotherapy course. If radiotherapy is recommended separately, your oncology teams will explain its purpose and timing. Do not assume it occurs on infusion or pump days.

### How long should I allow for the appointment?

The infusion itself is usually about an hour at first, but blood tests, review, pharmacy preparation and observation add time. Ask your unit for its expected visit length. Larger doses or a previous reaction may need a longer infusion.

> **Try explaining the plan back:** Can you explain which medicines you are receiving, the schedule, the contact plan and the symptoms that need prompt help?

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**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Day 1 · antibody monotherapy

**Active medicines:** Panitumumab

**Emetogenic risk:** Low

| Medicine | Dose / route | When |
|---|---|---|
| Ondansetron | 8 mg PO once | Before panitumumab, according to the individual antiemetic prescription<br>NCIS V8 low-risk row. Routine infusion-reaction premedication is a separate issue and is not usually required for panitumumab. |

| Medicine | Dose / route | When |
|---|---|---|
| Metoclopramide | 10 mg PO up to three times daily as required | Rescue nausea treatment only as prescribed<br>NCIS V8 rescue option; check contraindications, renal function, interactions and prescribed duration. |

## Additional phase-specific support

- Antiemetic classification refers to the active treatment day. Reconcile duplicate steroid entries before prescribing.
- No automatic granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) schedule is added; use the patient's neutropenia history, risk and consultant plan.
- No routine cetuximab-style antihistamine/steroid premedication is imported into panitumumab.
- Provide local EGFR skin care, periodic magnesium/calcium/potassium checks and monthly post-treatment electrolytes for two months.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

# What to watch for

You do not need to know which medicine caused a symptom before calling.

### 1 Skin / rash

**Panitumumab**

Report rash, cracks, painful or infected skin early.

### 2 Eyes

**Panitumumab**

New eye pain, light sensitivity or blurred vision: prompt advice.

### 3 Breathing

**Panitumumab**

New or worsening breathlessness or cough needs assessment.

### 4 Bowel

**Panitumumab**

Severe diarrhoea, dizziness or inability to drink: urgent contact.

### 5 Nails

**Panitumumab**

Painful or infected nail folds need early treatment.

### 6 Blood minerals

**Panitumumab**

Tests check magnesium, calcium and potassium; report cramps or weakness.

### 7 Hydration

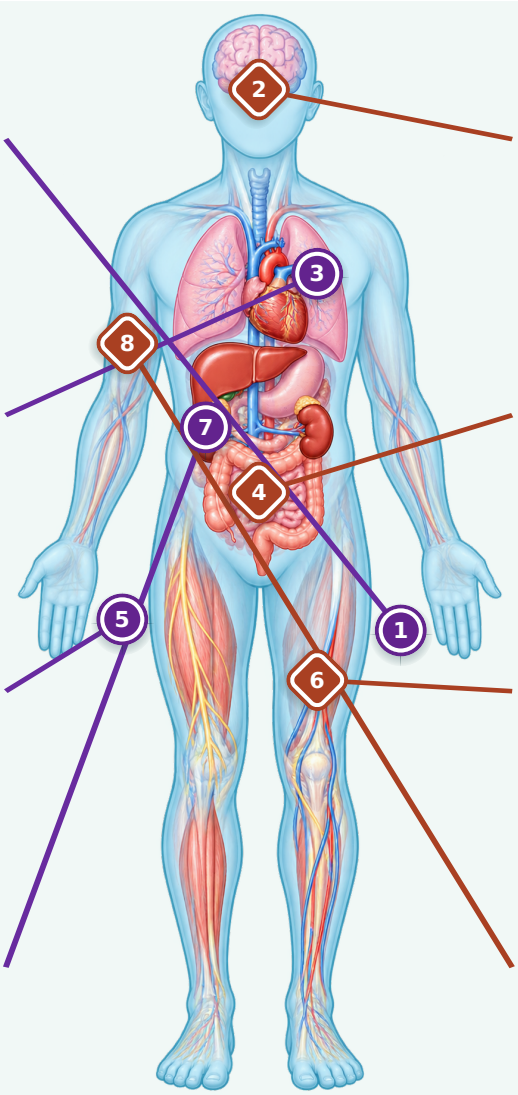
**Panitumumab**

Reduced urine or severe dehydration needs urgent advice.

### 8 Infusion reaction

**Panitumumab**

Tell the nurse about swelling, itching, flushing or breathing changes.




Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Report new breathing difficulty, eye pain or blurred vision, blistering or infected skin, severe diarrhoea or dehydration promptly.

**When to get help**

**Call 112 / 999** for severe breathing difficulty, severe chest pain, collapse or a seizure.

**Call oncology immediately:** temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

SACTCHECK · CHAPTER 04

# Your record and contact card

Panitumumab · National Cancer Control Programme (NCCP) 00225

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

## Panitumumab · EGFR-targeted antibody

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Skin / nail changes** | □ None | □ Mild change: report | □ Pain or daily tasks affected: call | □ Spreading infection or blistering: urgent review | □ Severe illness / collapse: emergency help |
| **Eye symptoms** | □ None | □ New irritation: report | □ Pain, redness or light sensitivity: call promptly | □ Blurred vision / severe eye pain: urgent review | □ Sudden major vision loss: emergency assessment |
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

# Keep your treatment details to hand

**Advanced / metastatic colorectal cancer** · NCCP 00225 v6 · Document revised 5 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

**FRONT / CONTACTS**

## My Treatment Card

SACTCheck

Name: _____

**Panitumumab**

Panitumumab Day 1 every 14 days.

Phase: _____ Cycle plan: team to confirm

**Daytime (EXAMPLE)** 087 12345678

**Out of hours (EXAMPLE)** 087 87654321

**Unwell or fever? Call oncology now.**
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

**BACK / TREATMENT RECORD**

## My cycle record

Rolling record; the team confirms the total treatment plan.

| Cycle | Date / initials |
|---|---|
| | |
| | |
| | |
| | |

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

**FOLD centre — do not cut**

## Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

## Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

### iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

### Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

## Your local contacts and resources

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.34 · NCCP 00225 v6 · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

SACTCHECK · CHAPTER 05

# Sources and further information

Panitumumab · National Cancer Control Programme (NCCP) 00225

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.


1. **National Cancer Control Programme (NCCP) 00225 v6 · official regimen**
   https://healthservice.hse.ie/documents/6560/225_Panitumumab_6mgkg.pdf
2. **Panitumumab product information ↗**
   https://www.medicines.org.uk/emc/product/6178/smpc
3. **Panitumumab patient medicine leaflet**
   https://www.medicines.org.uk/emc/product/6178/pil
4. **HSE: chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
5. **20100007 primary report**
   https://pubmed.ncbi.nlm.nih.gov/27736842/
6. **NCCP NCIS supportive antiemetic medicines V8 (2025)**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
7. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
   https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq